PharmEasy Labs

Report for Saad Salim Mulla(27Y/M)

Tests asked Insfa, Comprehensive Plus Checkup With Vitamins And Electrolytes

Test date 26 Jan 2025 **Report status** Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience | NABL Accredited Labs | 100+ Crore Samples Processed

PharmEasy Labs

Name : SAAD SALIM MULLA(27Y/M)

Ref. By : SELF

ADDRESS :

A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

Report Availability Summary

Full Report Available

Note : This is summary page. Please refer to the table below for the details

Test	Report Status
COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES	Available
25-OH VITAMIN D (TOTAL)	Available
CHLORIDE	Available
COMPLETE URINE ANALYSIS	Available
FASTING BLOOD SUGAR(GLUCOSE)	Available
HbA1c	Available
HEMOGRAM - 6 PART (DIFF)	Available
IRON	Available
KIDPRO	Available
LIPID PROFILE	Available
LIVER FUNCTION TESTS	Available
SODIUM	Available
TOTAL IRON BINDING CAPACITY (TIBC)	Available
TOTAL THYROXINE (T4)	Available
TOTAL TRIIODOTHYRONINE (T3)	Available
TSH - ULTRASENSITIVE	Available
UNSAT.IRON-BINDING CAPACITY(UIBC)	Available
VITAMIN B-12	Available
INSULIN - FASTING	Available

Note : Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by : Thyrocare Technologies Ltd.

PharmEasy Labs

NAME	: SAAD SALIM MULLA(27Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)		

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	73.56	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	: 26 Jan 2025 09:16
Sample Received on (SRT)	: 26 Jan 2025 12:23
Report Released on (RRT)	: 26 Jan 2025 12:58
Sample Type	: FLUORIDE PLASMA
Labcode	: 2601071796/PE002
Barcode	: DI664852

Dr Vitthal Kendre MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: 1st floor, Avirahi Arcade, Kandivali (W), Mumbai - 400067
7022 000 900

PharmEasy Labs

NAME : SAAD SALIM MULLA(27Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)

HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	5.5	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	111	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 26 Jan 2025 09:16
Sample Received on (SRT) : 26 Jan 2025 12:24
Report Released on (RRT) : 26 Jan 2025 13:42
Sample Type : EDTA Whole Blood
Labcode : 2601071969/PE002
Barcode : DJ840952

Dr Vitthal Kendre MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

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NAME : SAAD SALIM MULLA(27Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)

HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD
EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	14.7	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	46.6	%	40.0-50.0
Total RBC	HF & EI	5.32	X 10^6/µL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	87.6	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	27.6	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.5	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	40.7	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.7	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	209.1	-	*Refer Note below
MENTZER INDEX	Calculated	16.5	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.93	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	50.8	%	40-80
Lymphocytes Percentage	**Flow Cytometry**	**43.6**	**%**	**20-40**
Monocytes Percentage	Flow Cytometry	3.3	%	2-10
Eosinophils Percentage	Flow Cytometry	1.6	%	1-6
Basophils Percentage	Flow Cytometry	0.4	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.52	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	**Calculated**	**3.02**	**X 10^3 / µL**	**1.0-3.0**
Monocytes - Absolute Count	Calculated	0.23	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.11	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	298	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	9.6	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	10.3	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	21.4	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.29	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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REF. BY	: SELF	A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)		

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**5.86**	**ng/mL**

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Sample Collected on (SCT)	: 26 Jan 2025 09:16
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Report Released on (RRT)	: 26 Jan 2025 18:34
Sample Type	: SERUM
Labcode	: 2601083849/PE002
Barcode	: DK279144

Dr Renuka MD(Path)

Dr Arshiya MD(Path)

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TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)		

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	214	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)		

TEST NAME	TECHNOLOGY	VALUE	UNITS
INSULIN - FASTING	C.L.I.A	13.78	µU/mL

Bio. Ref. Interval. :-

1.9-23 µU/mL

Clinical Significance

Type I (Insulin dependent: "Juvenile") diabetes is due to a destruction of the beta cells, with a consequence of absolute lack of insulin. In type II (Non insulin-dependent: "Maturity onset") diabetes, insulin resistance may play an important role; However after several years of evolution, beta-cells failure may occur, leading to a relative insulinopenia requiring, in some cases, insulin administration. Insulin resistance is associated with high circulation levels of the hormone.

For diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision: Intra Assay (%CV): 4.20 %, Inter Assay (%CV): 5.60%; Sensitivity: 0.03 µU/mL

External quality control program participation:

College Of American Pathologists: Insulin Survey (Ing): Cap Number: 7193855-01

Kit validation references:

Howanitz PJ, Howanitz JH, Henry JB. Carbohydrates.Clinical Diagnosis and Management by Laboratory Methods 1991 ;172-182.edited by Henry JB, Philadelphia, W.B Saunders Company.

Please correlate with clinical conditions.

Method:- One step Immunoenzymatic (Sandwich) assay.

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HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	110	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 **Method :** Ferrozine method without deproteinization			
TOTAL IRON BINDING CAPACITY (TIBC)	PHOTOMETRY	430	µg/dL
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay			
% TRANSFERRIN SATURATION	CALCULATED	26	%
Bio. Ref. Interval. : 13 - 45 **Method :** Derived from IRON and TIBC values			
UNSAT.IRON-BINDING CAPACITY(UIBC)	PHOTOMETRY	320.4	µg/dL
Bio. Ref. Interval. : 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY			

Please correlate with clinical conditions.

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HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	126	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**29**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	91.1	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	98	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.4	Ratio	3 - 5
TRIG / HDL RATIO	**CALCULATED**	**3.42**	**Ratio**	**< 3.12**
LDL / HDL RATIO	CALCULATED	3.2	Ratio	1.5-3.5
HDL / LDL RATIO	**CALCULATED**	**0.31**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	CALCULATED	97.02	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	19.54	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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Lab Address: D-37/1. TTC MIDC, Turbhe. Navi Mumbai, 400703
7022 000 900

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NAME : SAAD SALIM MULLA(27Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)

HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	74.1	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.94	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.21	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.73	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	24.5	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	31.7	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	**PHOTOMETRY**	**49**	**U/L**	**< 45**
SGOT / SGPT RATIO	CALCULATED	0.65	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.31	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.32	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	2.99	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.44	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	I.S.E - INDIRECT	140.9	mmol/L

Bio. Ref. Interval. :

Adults: 136-145 mmol/l

Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	I.S.E - INDIRECT	103.1	mmol/L

Bio. Ref. Interval. :

ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	10.37	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.86	mg/dL	0.72-1.18
BUN / Sr.CREATININE RATIO	CALCULATED	12.06	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	22.19	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	25.8	Ratio	< 52
CALCIUM	PHOTOMETRY	9.16	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**7.6**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: D-37/1. TTC MIDC, Turbhe. Navi Mumbai, 400703
7022 000 900

PharmEasy Labs

NAME : SAAD SALIM MULLA(27Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)

HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST
MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	153	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	11.7	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	2.22	µIU/mL	0.54-5.30

Comments : SUGGESTING THYRONORMALCY

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Fully Automated Electrochemiluminescence Compititive Immunoassay
T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference
In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT) : 26 Jan 2025 09:16
Sample Received on (SRT) : 26 Jan 2025 14:52
Report Released on (RRT) : 26 Jan 2025 18:34
Sample Type : SERUM
Labcode : 2601083849/PE002
Barcode : DK279144

Dr Renuka MD(Path)

Dr Arshiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs

NAME	: SAAD SALIM MULLA(27Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)		

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	119	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- CKD-EPI Creatinine Equation

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NAME : SAAD SALIM MULLA(27Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,INSULIN (F)

HOME COLLECTION :
A/2 206 HUMERA PARK PATHANWADI MALAD EAST MUMBAI - 400097 MALAD EAST MUMBAI

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	Yellow	-	Pale Yellow
APPEARANCE	**Visual Determination**	**SLIGHT CLOUDY**	**-**	**Clear**
SPECIFIC GRAVITY	pKa change	> 1.030	-	1.003-1.030
PH	pH indicator	5.5	-	5-8
Chemical Examination				
URINARY PROTEIN	**PEI**	**Trace (15-30 mg/dl)**	**mg/dL**	**Absent**
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	1	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	5	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	5	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	**Microscopy**	**PRESENT**	**-**	**Absent**
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

Remarks : Alert!!! Spermatozoa are seen.

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

~~ End of report ~~

Sample Collected on (SCT) : 26 Jan 2025 09:16
Sample Received on (SRT) : 26 Jan 2025 13:40
Report Released on (RRT) : 26 Jan 2025 15:32
Sample Type : URINE
Labcode : 2601078661/PE002
Barcode : DE441092

Dr Vitthal Kendre MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

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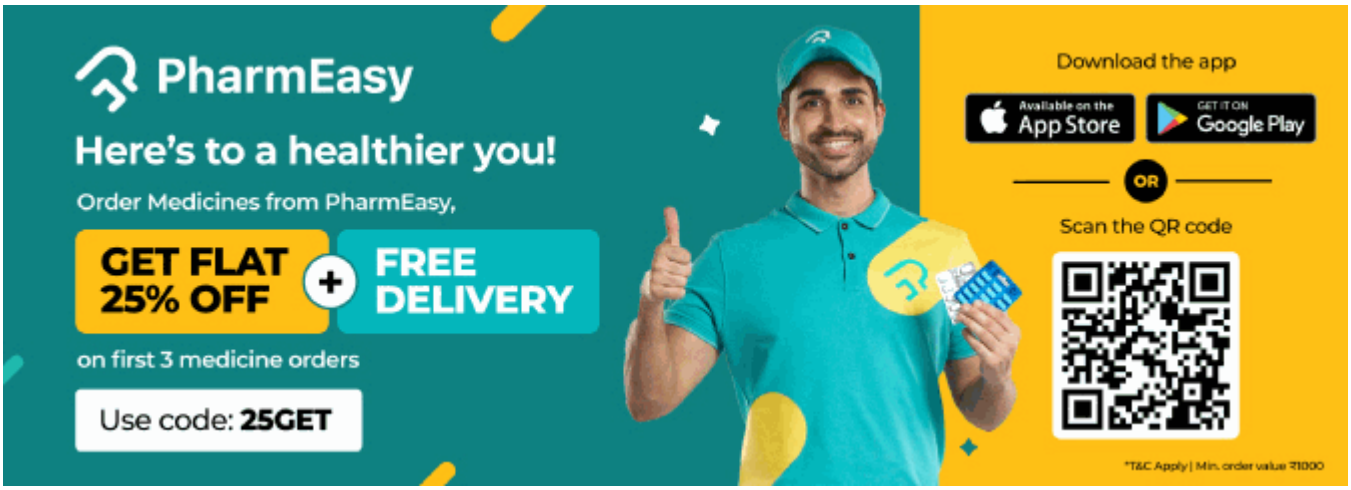
Lab Address: 1st floor, Avirahi Arcade, Kandivali (W), Mumbai - 400067
7022 000 900

CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.